National Cancer Control Programme (NCCP) 00861 · clinician draft

Pembrolizumab 400 mg + weekly carboplatin AUC 1.5 / paclitaxel → dose-dense AC

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

After surgery · pembrolizumab alone

Active medicines: Pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- After surgery: pembrolizumab 400 mg every 42 days for five doses, beginning six weeks after cycle 8 Day 1, subject to recovery, surgery and the oncology plan. Progression preventing definitive surgery or unacceptable pembrolizumab toxicity changes this plan; postoperative treatment is not automatic.
- No routine cytotoxic antiemetics, taxane premedicines or chemotherapy granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) are carried forward.
- Continue the immune-toxicity and required blood/hormone monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00861 v2 · p9 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.13 · Regimen-level drafts; prescriber and oncology-pharmacy verification required